

Data reveal how doctors take women's pain less seriously than men's

A study of hospital emergency departments suggests that women have more limited access to painkillers and medical care.

By [Lilly Tozer](#)



Women are less likely than men to be given painkillers. Credit: Natalia Gdovskaia/Getty

Physicians treat men and women differently when it comes to pain — women in hospital wait longer to be seen and are less likely to receive pain medication than men, finds a study comparing how pain is perceived and treated in male and female patients.

The findings, published on 5 August in *Proceedings of the National Academy of Sciences*¹, highlight how our perception of others' experiences of pain can be affected by unconscious bias.

“Women are viewed as exaggerating or hysterical and men are viewed as more stoic when they complain of pain,” says co-author Alex Gileles-Hillel, a physician-scientist at Hadassah-Hebrew University Medical Center in Jerusalem.

Minimizing women's pain

Gileles-Hillel and his colleagues investigated the extent of this bias at emergency departments in Israeli and US hospitals. They analysed more than 20,000 discharge notes of patients who had come in with 'non-specific' pain complaints — those without a clear underlying cause — such as headaches.

The analysis found that, when first arriving at hospital, women were 10% less likely than men to have a recorded pain score — a number from 1 to 10, given by the patient, that helps to inform physicians about the severity of pain. After the initial assessment, women waited an average of 30 minutes longer than men to see a physician, and were less likely than men to receive pain medication. This trend was consistent regardless of the gender of the nurse or doctor. "Women can hold the same stereotypical views as men about women's pain," says Gileles-Hillel.

The researchers also tested how 100 health-care professionals perceived patients' pain. Participants were presented with a scenario of a patient with a severe backache and were given the patient's previous clinical information. The patient profiles were identical, except for sex. Participants consistently gave higher pain scores to the male patient than to the female one.

"One of the reasons that we see this in the pain context is because there aren't objective measures for pain, so the physician has to rely on the reporting of the patient. That allows for more bias," says Diane Hoffmann, a health-care-law researcher at the University of Maryland in Baltimore. She adds that the issue should be highlighted during medical training, to equip physicians with a better understanding of pain and the potential for bias when treating it.

A more immediate solution that Gileles-Hillel wants to test is whether using computer systems to generate reminders could be enough to improve fairness — for example, an alert could advise a doctor to prescribe painkillers when a patient has reported a high pain score, regardless of gender. "Physicians are not aware of this bias," he says. "Raising awareness is one solution."

doi: <https://doi.org/10.1038/d41586-024-02547-7>

References

1. Guzikevits, M. *et al. Proc. Natl Acad. Sci. USA* **121**, e2401331121 (2024).